

Discharge Summary

Dear Colleagues,

Ms. Erika Lange, DOB 06/14/1958, was admitted on 07/22/2024 for inpatient structured diabetes education and therapy optimization. She was diagnosed with type 2 diabetes 11 years ago and has required progressive escalation of therapy. At admission her regimen was metformin 2000 mg daily, empagliflozin 10 mg daily, and saxagliptin 5 mg daily, with self-monitored glucose values consistently in the range of 240–310 mg/dL.

Primary Diagnosis: Type 2 diabetes mellitus with multiple complications (E11.7)

Laboratory values during the inpatient stay:

Parameter	Value	Reference
HbA1c	11.2%	< 7.0%
Fasting glucose (admission)	312 mg/dL	70–100 mg/dL
Fasting glucose (discharge)	142 mg/dL	70–100 mg/dL
Triglycerides	284 mg/dL	< 150 mg/dL

Past Medical History: Ms. Lange has had arterial hypertension since 2012, on telmisartan 80 mg and amlodipine 5 mg. Non-alcoholic fatty liver disease diagnosed 2020, followed by hepatology annually. Mild non-proliferative diabetic retinopathy identified at ophthalmology in 2023 — no treatment required. BMI 34.2 kg/m2. Lifelong non-smoker, no alcohol. Works full-time as a nurse on rotating shifts — attributed poor glycemic control in part to irregular meal patterns.

Physical Examination on Admission: Height 163 cm, weight 91 kg. BP 144/86 mmHg, HR 80 bpm. Monofilament sensation mildly reduced in both feet but no pressure points, deformity, or wounds. No xanthomata. Fundoscopy deferred to outpatient.

Hospital Course: Empagliflozin and saxagliptin discontinued. Basal insulin glargine 12 units at bedtime initiated and uptitrated to 16 units by day 7. Structured group diabetes education over 5 days covered carbohydrate counting, injection technique, hypoglycaemia management, sick day rules, and foot care. Dietitian provided individualised meal planning accounting for shift work. Flash CGM initiated day 3 with sensor placement and interpretation training.

Interdisciplinary Team: Podiatrist confirmed intact skin, documented reduced monofilament sensation, and emphasised daily foot inspection. Diabetes educator assessed competencies at admission and discharge — Ms. Lange demonstrated proficiency in insulin self-administration, glucose monitoring, carbohydrate counting, and hypoglycaemia recognition prior to discharge.

Patient Engagement: Ms. Lange's engagement with the education programme was consistently high throughout the admission. Her confidence and self-efficacy scores improved significantly from day 1 to day 7 on the standardised assessment tool. She expressed particular concern about managing insulin during night shifts, which was addressed with a dedicated session and a written shift-work insulin guide.

Medication at Discharge: Metformin 1000 mg twice daily, telmisartan 80 mg, amlodipine 5 mg, insulin glargine 16 units at bedtime, pantoprazole 20 mg.

Follow-up: Diabetology in 4 weeks for insulin titration and CGM download. Ophthalmology in 3 months. Podiatry annual assessment arranged. HbA1c and renal panel in 3 months. GP in 2 weeks.

Structured Diabetes Education — Detailed Competency Assessment:

Ms. Lange attended five consecutive daily group education sessions, each 90 minutes long, facilitated by the diabetes specialist nurse and the registered dietitian. The sessions covered: pathophysiology of type 2 diabetes and rationale for insulin; insulin injection technique, rotation sites, storage, and sharps disposal; carbohydrate counting and individualised meal planning for shift workers; hypoglycaemia definition, recognition, treatment, and prevention; sick day rules and foot care.

A standardised competency checklist was completed at the end of the programme. Ms. Lange demonstrated full competency in all domains prior to discharge:

accurate carbohydrate calculation from food labels, correct insulin self-injection technique and site rotation, interpretation of CGM glucose traces and appropriate insulin adjustments, correct management of a simulated hypoglycaemia scenario, and accurate recitation of sick day rules. Her confidence and self-efficacy scores improved markedly from day 1 to day 7 on the standardised assessment tool used by the unit.

Dietitian Assessment and Shift Work Meal Planning:

The registered dietitian conducted an extended assessment on days 2 and 5. A 3-day dietary recall revealed high variability in carbohydrate intake between shift days and rest days, and a tendency to skip meals during night shifts followed by large caloric intake in the post-shift recovery period. An individualised meal plan was developed accounting for her rotating shift pattern, with specific guidance on carbohydrate distribution and strategies for maintaining adequate nutrition without meal skipping. A high-fibre calorie-appropriate diet was recommended with a target daily carbohydrate intake of 180–200 g distributed across three main meals and two snacks. The plan was provided in written form with worked examples for different shift types.

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